

AI-Powered HMS Synthetic Clinical Dataset

Clinical Note

Patient Name	Do Xuan Nam	MRN	MRN-0028
DOB	1962-11-21	Gender	Female
Encounter Date	2024-04-21	Department	Internal Medicine
Attending Provider	RN Jamie Owens	Status	active

Synthetic Data Warning: This document is generated for software testing, OCR parsing, chunking, embedding, access-control validation, and Graph RAG demonstrations. It is not valid medical advice and must not be used for patient care.

Subjective

Do Xuan Nam is a 63-year-old patient in Internal Medicine presenting for follow-up of fatigue, elevated home glucose, and intermittent dizziness. PMH includes T2DM, HTN, obesity, intermittent medication non-adherence. Patient reports symptoms are stable overall. Denies severe chest pain, syncope, active bleeding, or new focal neurologic deficit unless otherwise documented. Current meds include metformin 500mg BID; lisinopril 10mg daily; atorvastatin 20mg nightly. Allergies: NKDA.

Objective

Vitals: BP 138/68 mmHg, HR 90 bpm, RR 18/min, Temp 36.4 C, SpO2 92% RA, weight 83 kg. Exam: alert, oriented, no acute distress. Cardiopulmonary exam without unstable findings. Abdomen soft, non-tender. Extremities show no acute deformity; edema varies by department-specific condition. Labs reviewed below; medication list reconciled.

Recent Lab Snapshot

Date	Analyte	Value	Unit	Reference Range	Status
2026-05-08	Creatinine	1.71	mg/dL	0.7-1.3	High
2026-05-08	eGFR	61.0	mL/min/1.73m2	>=60	Normal
2026-05-08	Glucose	125.0	mg/dL	70-99	High
2026-05-08	HbA1c	7.2	%	<5.7	High
2026-05-08	BNP	202.0	pg/mL	<100	High
2026-05-08	AST	29.0	U/L	10-40	Normal
2026-05-08	ALT	34.0	U/L	7-56	Normal
2026-05-08	Potassium	4.4	mmol/L	3.5-5.0	Normal
2026-05-08	Hemoglobin	13.3	g/dL	12.0-17.5	Normal

Assessment

Primary assessment: type 2 diabetes mellitus, hypertension, chronic kidney disease risk. Patient is clinically stable for ongoing management. Risk considerations include renal dosing, medication safety, fall risk, and need for timely escalation if red-flag symptoms occur.

Plan

Check HbA1c and renal panel. Reinforce diet, activity, and home BP log. Review medication adherence and side effects. Provide patient education, update care team via SBAR, and document any access-relevant clinical justification before AI-assisted retrieval of PHI.

Detailed Care Coordination Notes

Problem List

1. type 2 diabetes mellitus, hypertension, chronic kidney disease risk. 2. Medication safety monitoring. 3. Functional status and discharge readiness. 4. Follow-up coordination with Internal Medicine.

Safety Triggers

Escalate to responsible clinician for SBP < 90, HR > 130, SpO2 < 92%, acute mental status change, active bleeding, severe pain, fever with neutropenia risk, or rapidly rising Cr/K.

AI Retrieval Tags

access_tags: read, summary, labs, medication, cardiology as applicable. Suggested document_type for ingestion: clinical_note.